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Health, Human Rights & Intervention (Edexcel Paper 2)

This topic explores the complex relationships between global health outcomes, human rights, and the role of geopolitical intervention. It examines how health varies globally, the causes and consequences of human rights abuses, the effectiveness and ethics of different forms of intervention by governments, IGOs, and NGOs, and how global governance structures shape health and rights outcomes across different scales.

1. Global Health Patterns

Health (WHO definition): A state of complete physical, mental, and social well-being, not merely the absence of disease or infirmity. This holistic definition highlights that health encompasses far more than the simple absence of illness — it includes psychological welfare, social functioning, and quality of life. Health outcomes are profoundly unequal globally, reflecting deep structural inequalities in wealth, governance, education, gender relations, and access to resources.

Measuring Health

No single indicator captures the full picture of a population's health. Geographers use multiple measures, each with strengths and limitations:

Indicator	Description	Strengths	Limitations
Life expectancy at birth	Average number of years a newborn can expect to live based on current mortality rates	Easy to compare internationally; widely available data; sensitive to overall development	Masks internal inequalities (gender, regional, ethnic, socioeconomic). A country with LE of 75 might have wealthy citizens living to 85 and poor citizens dying at 60

Infant mortality rate (IMR)	Deaths of infants under 1 year per 1,000 live births	Very sensitive indicator of healthcare quality, maternal health, sanitation, and nutrition. Closely correlated with development level	Can be affected by different national recording methods (e.g. whether stillbirths are counted as live births varies). Does not reveal causes of death
Under-5 mortality rate (U5MR)	Deaths of children under 5 per 1,000 live births	Captures risks beyond infancy (malnutrition, infectious diseases, lack of vaccination). A key MDG/SDG indicator	Does not reveal specific causes; data quality varies in LICs where many deaths go unregistered
Maternal mortality ratio (MMR)	Deaths of women during pregnancy, childbirth, or within 42 days of delivery, per 100,000 live births	Powerful indicator of women's access to healthcare, skilled birth attendants, and gender equality. Enormous global variation (e.g. Chad ~1,140 vs Norway ~2)	Significantly under-reported in many LICs due to home births and poor death registration
DALYs	Disability-Adjusted Life Years — years of healthy life lost to premature death, illness, or disability	Captures both fatal and non-fatal disease burden. Allows comparison of very different diseases (e.g. malaria vs depression)	Complex to calculate; relies on subjective disability weightings; significant data gaps in LICs
HALE	Healthy Life Expectancy — average number of years lived in full health (adjusts LE for time spent in ill health)	More meaningful than raw LE because quality of life matters, not just length. Japan has the highest HALE globally (~74 years)	Relies on self-reported health data and subjective assessments of disability severity

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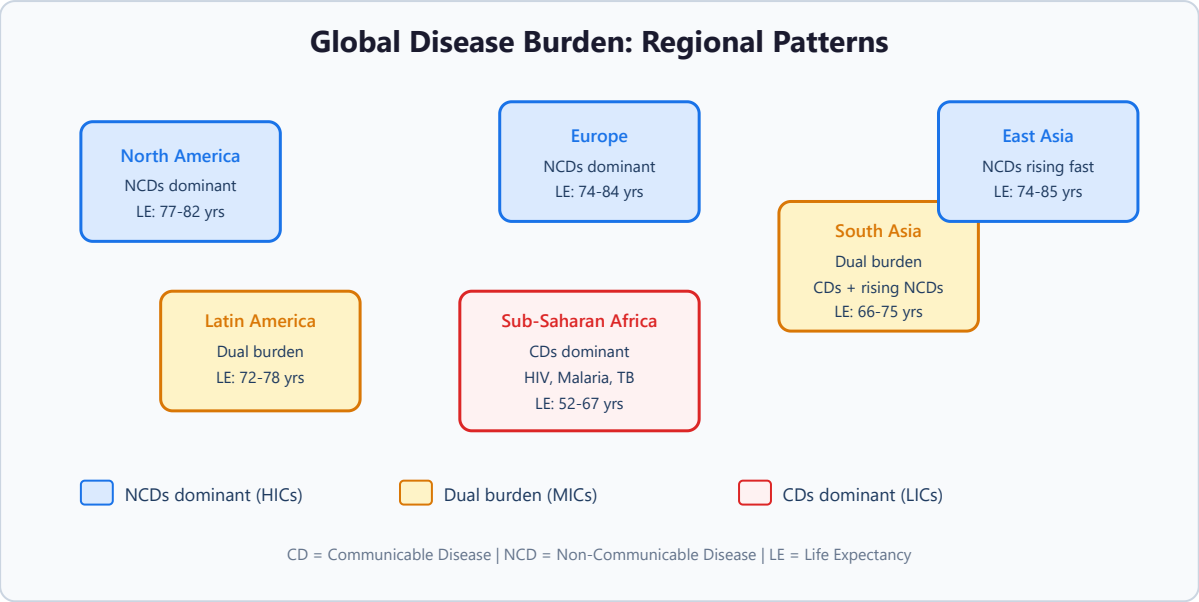
Exam Tip: When asked to evaluate health indicators, always discuss both strengths AND limitations. A top-level answer will explain that **composite indicators** (HALE, DALYs) are more useful than simple measures (IMR, LE) because they capture multiple dimensions of health, but they require more data and are harder to calculate accurately in data-poor countries.

Global Patterns of Health

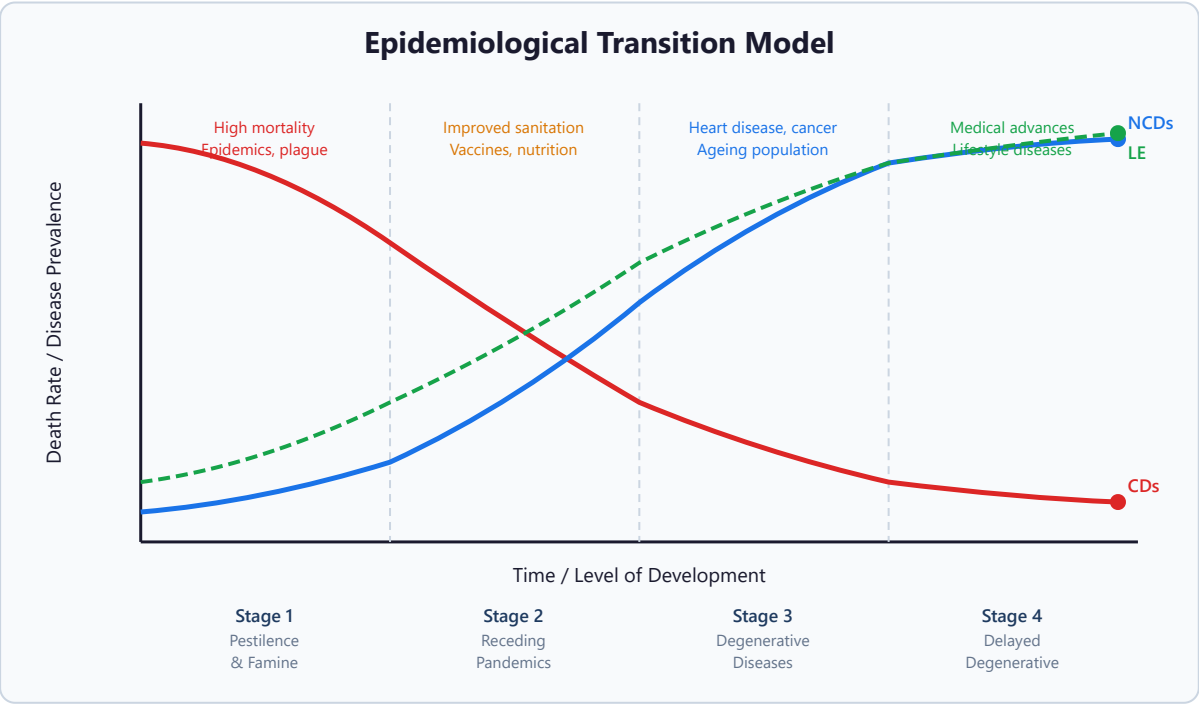
Health outcomes show clear global patterns linked to development levels, though with significant exceptions and within-country variation:

- **High-income countries (HICs):** Life expectancy typically 78–85+ years; IMR <5 per 1,000; disease burden dominated by **non-communicable diseases (NCDs)** such as cardiovascular disease, cancers, diabetes, and mental health conditions. Ageing populations create growing healthcare demand. Examples: Japan (LE 84), UK (LE 81), USA (LE 77 — lower than expected for its wealth due to inequality, gun violence, opioid crisis, and high obesity rates)
- **Middle-income countries (MICs):** Rapidly improving but highly uneven outcomes. Face a **dual burden** of communicable AND non-communicable diseases simultaneously (e.g. HIV/TB alongside rising diabetes/heart disease). LE 65–78 years. Examples: India (LE 70, huge regional variation), Brazil (LE 76, significant racial disparities), China (LE 78, rapid improvement since 1980)
- **Low-income countries (LICs):** LE often 50–65 years; IMR >40 per 1,000 in worst cases; disease burden dominated by **communicable diseases** (malaria, TB, HIV/AIDS, diarrhoeal diseases, respiratory infections). Maternal and child mortality remain extremely high. Examples: Chad (LE 54), Central African Republic (LE 54), Sierra Leone (LE 55)



2. The Epidemiological Transition Model

Epidemiological transition: The shift in the pattern of disease and causes of death that accompanies economic development and demographic change. First proposed by Abdel Omran (1971), it describes how populations transition from predominantly infectious/communicable disease mortality to chronic/degenerative non-communicable disease mortality as they develop, urbanise, and age. The model parallels the demographic transition model.



Stages of the Epidemiological Transition

Stage	Name	Characteristics	Example Countries
1	Age of Pestilence and Famine	Very high mortality from infectious diseases, famine, and warfare. Life expectancy 20–40 years. Periodic devastating epidemics (plague, cholera, smallpox). High infant and child mortality (>200 per 1,000). Population growth slow or stagnant due to high death rate.	Pre-industrial societies. Some elements still present in conflict-affected LICs (South Sudan, parts of DR Congo)
2	Age of Receding Pandemics	Improved nutrition, sanitation, clean water supply, and early medicine (vaccines, antibiotics) reduce infectious disease mortality. Life expectancy rises to 50–60 years. Mortality becomes more predictable and less subject to catastrophic epidemics. Population growth accelerates as death rates fall faster than birth rates.	19th-century Europe. Present-day Bangladesh (LE 73), Kenya (LE 62), Ethiopia (LE 67)
3	Age of Degenerative and Man-Made Diseases	NCDs (cardiovascular disease, cancer, stroke, diabetes) become the leading causes of death, reflecting ageing populations, sedentary lifestyles, processed diets, tobacco and alcohol use, and industrial pollution. Life expectancy 65–75+. Communicable diseases largely controlled but not eliminated.	Present-day China (rapid transition), Brazil, Turkey, many MICs undergoing urbanisation and dietary change
4	Age of Delayed Degenerative Diseases	Advanced medical technology (surgical intervention, pharmaceuticals, screening programmes) delays mortality from degenerative diseases. Rising importance of obesity, mental health, substance misuse, and emerging infections (e.g.	UK, Japan, USA, Western Europe, Australia

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		antimicrobial resistance). Life expectancy 78–85+.	
5 (proposed)	Age of Re-emerging Infections and Societal Diseases	Some scholars propose a fifth stage characterised by: antimicrobial resistance (AMR) making previously treatable infections lethal again; emergence of novel pandemics (COVID-19, SARS, Ebola); persistent lifestyle-related conditions; rising mental health burden; and widening health inequalities even within HICs.	Global — COVID-19 pandemic demonstrated that all countries remain vulnerable to new infectious diseases regardless of development level

Exam Tip: The epidemiological transition model is useful but must be critiqued. Key criticisms: (1) it assumes a **linear progression** that all countries follow, ignoring the **dual burden** experienced by many MICs facing communicable AND non-communicable diseases simultaneously, (2) it was based primarily on **Western European historical experience** and may not apply to countries following different development paths, (3) it underestimates the persistence of infectious diseases in LICs and the emergence of new pandemics (COVID-19 challenged the assumption that infectious diseases only dominate in early stages), (4) it does not adequately account for within-country inequalities (e.g. Indigenous Australians have significantly lower LE than non-Indigenous Australians despite Australia being a HIC). Always evaluate models rather than simply describing them.

3. Communicable vs Non-Communicable Diseases

Communicable diseases (CDs): Diseases caused by pathogens (bacteria, viruses, parasites, fungi) that can be transmitted between individuals, directly (contact, droplets, bodily fluids) or indirectly (vectors, contaminated water/food, fomites). They are disproportionately concentrated in LICs and tropical regions due to poverty, poor sanitation, lack of healthcare access, and vector-friendly climates.

Non-communicable diseases (NCDs): Diseases that are not transmitted between people. They tend to be chronic (long-lasting), progressive, and linked to lifestyle factors (diet, exercise, smoking, alcohol), genetic predisposition, environmental pollution, and ageing. The four main NCDs —

cardiovascular diseases, cancers, chronic respiratory diseases, and diabetes — account for over 70% of all deaths globally and are the leading causes of death in every world region including Africa.

Feature	Communicable Diseases	Non-Communicable Diseases
Cause	Pathogens (bacteria, viruses, parasites, fungi)	Lifestyle, genetics, environmental factors, ageing
Transmission	Person to person, vectors, contaminated water/food/surfaces	Not transmissible between people (though risk factors can be shared, e.g. family diet)
Duration	Usually acute (short-term), though some are chronic (HIV, TB, hepatitis)	Usually chronic (long-term, often lifelong)
Prevention	Vaccination, sanitation improvement, vector control, hygiene education, antibiotics	Lifestyle changes (diet, exercise, smoking cessation), screening, regulation (sugar tax, smoking bans), early detection
Geographic pattern	Higher burden in LICs, tropical regions, conflict zones, areas with poor infrastructure	Higher burden in HICs; rising very rapidly in MICs undergoing nutrition and lifestyle transitions
Key examples	Malaria, HIV/AIDS, TB, cholera, Ebola, COVID-19	Heart disease, stroke, cancer (lung, breast, colorectal), type 2 diabetes, COPD, depression
Economic impact	Loss of productive workforce (HIV in Sub-Saharan Africa); healthcare costs; reduced agricultural output	Long-term healthcare costs (the NHS spends ~70% of its budget on long-term conditions); reduced productivity; care burden on families

Case Study — HIV/AIDS in Sub-Saharan Africa: Sub-Saharan Africa contains approximately **65%** of all people living with HIV globally (~25 million people), despite having only ~17% of the world's population. Eswatini (formerly Swaziland) has the highest adult prevalence at approximately **27%**. Contributing factors include: poverty (inability to access healthcare or condoms), gender inequality (women have limited power to negotiate safe sex), cultural stigma surrounding HIV testing and

treatment, weak healthcare infrastructure, high rates of concurrent sexual partnerships, and historical denial by political leaders (e.g. Thabo Mbeki in South Africa in the early 2000s, who questioned the link between HIV and AIDS, delaying the rollout of antiretroviral therapy and resulting in an estimated 330,000 preventable deaths). **Interventions:** PEPFAR (US President's Emergency Plan for AIDS Relief, the largest single-disease programme in history, providing $100+ billion since 2003 for prevention, treatment, and care); the Global Fund to Fight AIDS, TB and Malaria; local NGO education campaigns; condom distribution. **Success story:** Botswana has achieved near-universal access to ART, raising life expectancy from 49 years (2002) to over 69 years (2023) — demonstrating that political will, international funding, and effective health systems can transform outcomes even in severely affected countries.

Case Study — NCDs in the UK: Cardiovascular disease and cancer are the leading causes of death in the UK, together responsible for over **50%** of all deaths. Risk factors include: obesity (approximately 28% of UK adults are obese, one of the highest rates in Europe), physical inactivity (only 66% of adults meet recommended activity levels), smoking (13% of adults, down from 45% in the 1970s), alcohol consumption (over 7 million people drink at levels that increase health risks), and an ageing population (18.6% aged 65+). The NHS spends an estimated **70%+** of its budget managing long-term conditions. Government interventions: the Soft Drinks Industry Levy (sugar tax, 2018) — reduced sugar content in drinks by 44% before it even took effect as manufacturers reformulated; indoor smoking ban (2007) reduced heart attack admissions by 2.4%; Change4Life and 5-a-day campaigns; national cancer screening programmes (breast, cervical, bowel); minimum unit pricing for alcohol in Scotland.

4. The Health-Development Link

Health and development are intrinsically linked in a **reinforcing cycle**. Poor health constrains economic development (reduced workforce productivity, increased healthcare spending, lower educational attainment for sick children), while poverty perpetuates poor health (inability to afford healthcare, clean water, nutritious food, sanitation). The relationship between GNI per capita and life expectancy is **logarithmic** — health gains per dollar of income are greatest at lower income levels and diminish at higher incomes. This means that relatively small increases in income in LICs can produce dramatic health improvements, while large increases in spending in HICs yield only marginal gains.

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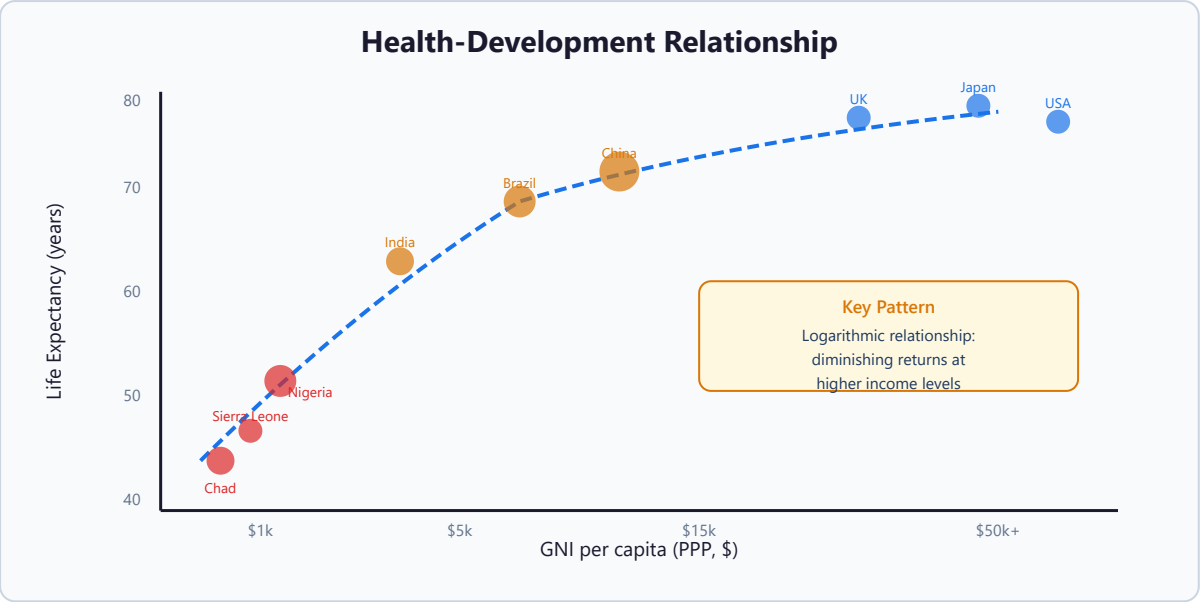


Factors Affecting Global Health Patterns

- **Wealth and income:** Higher GNI per capita enables better nutrition, sanitation infrastructure, healthcare spending, and disease prevention. The relationship is logarithmic — initial gains from escaping extreme poverty are enormous (clean water, basic vaccination), while additional spending in wealthy countries yields smaller marginal improvements. However, the USA demonstrates that wealth alone does not guarantee good health — inequality, lifestyle factors, and healthcare system design also matter greatly
- **Education (especially female education):** One of the strongest predictors of child survival worldwide. Educated mothers are more likely to: seek prenatal care, deliver in a health facility with a skilled attendant, vaccinate their children, practice good hygiene and nutrition, use family planning, and earn income to invest in family health. UNESCO estimates that if all women completed secondary education, child mortality in Sub-Saharan Africa would fall by 49%
- **Governance and political stability:** Stable, transparent, democratic governance enables effective health systems, disease surveillance, and resource allocation. Corruption diverts healthcare resources (the WHO estimates corruption accounts for $500 billion lost annually in health sectors globally). Armed conflict destroys health infrastructure, displaces populations, disrupts supply chains, and creates conditions for disease outbreaks
- **Climate and physical environment:** Tropical climates harbour disease vectors (mosquitoes for malaria, dengue, Zika; tsetse flies for sleeping sickness). Water scarcity and poor sanitation increase waterborne diseases (cholera, typhoid). Climate change is expanding the range of vector-borne diseases into

previously unaffected areas and increasing the frequency of extreme weather events that damage health infrastructure

- **Globalisation:** A double-edged sword for health. Facilitates the rapid spread of infectious diseases across borders (pandemics travel via air routes). But also enables transfer of medical knowledge, pharmaceutical trade, international health cooperation, and economic growth that funds health improvements
- **Culture, gender, and social norms:** Cultural practices can positively or negatively affect health-seeking behaviour. Gender inequality limits women's access to healthcare, education, and nutrition in many societies (e.g. female genital mutilation, child marriage, restricted mobility). Social stigma around mental illness, HIV, and disability prevents people from seeking treatment

5. Human Rights

Human rights: Universal, inalienable rights and freedoms to which all people are entitled, regardless of nationality, sex, ethnicity, religion, language, or other status. The foundational document is the **Universal Declaration of Human Rights (UDHR)**, adopted by the UN General Assembly on 10 December 1948 (in response to the atrocities of World War II and the Holocaust). It sets out 30 fundamental rights, including the right to life, liberty, education, health, freedom from torture, and freedom of expression. While not legally binding itself, the UDHR has inspired legally binding international treaties including the International Covenant on Civil and Political Rights (ICCPR) and the International Covenant on Economic, Social and Cultural Rights (ICESCR).

Key Human Rights Issues in Geography

- **Right to health (Article 25 UDHR):** Access to healthcare, clean water, adequate nutrition, and sanitation. Violated when governments fail to provide basic health services or when healthcare is prohibitively expensive for the poor
- **Right to education (Article 26):** Free and compulsory primary education; equal access to higher education. Still denied to ~244 million children globally, disproportionately girls in LICs
- **Freedom from torture (Article 5):** Prohibition of cruel, inhuman, or degrading treatment. Violated systematically in authoritarian states, conflict zones, and detention facilities
- **Freedom of expression (Article 19):** Right to hold opinions and express them without interference. Restricted in authoritarian regimes through censorship, imprisonment of journalists, and internet

shutdowns

- **Right to asylum (Article 14):** Protection from persecution in other countries. Challenged by rising anti-immigration sentiment and border barriers in many HICs
- **Gender equality:** Equal rights regardless of sex; freedom from discrimination. Violated through FGM (affecting 200+ million women globally), forced/child marriage, honour-based violence, and unequal legal rights
- **Children's rights (UN Convention on the Rights of the Child, 1989):** Protection from exploitation, child labour (152 million children in child labour globally), trafficking, and recruitment as child soldiers (~250,000 worldwide)

Patterns of Human Rights Violations

Human rights abuses are not confined to any single region but are concentrated where conflict, authoritarian governance, poverty, and discrimination intersect:

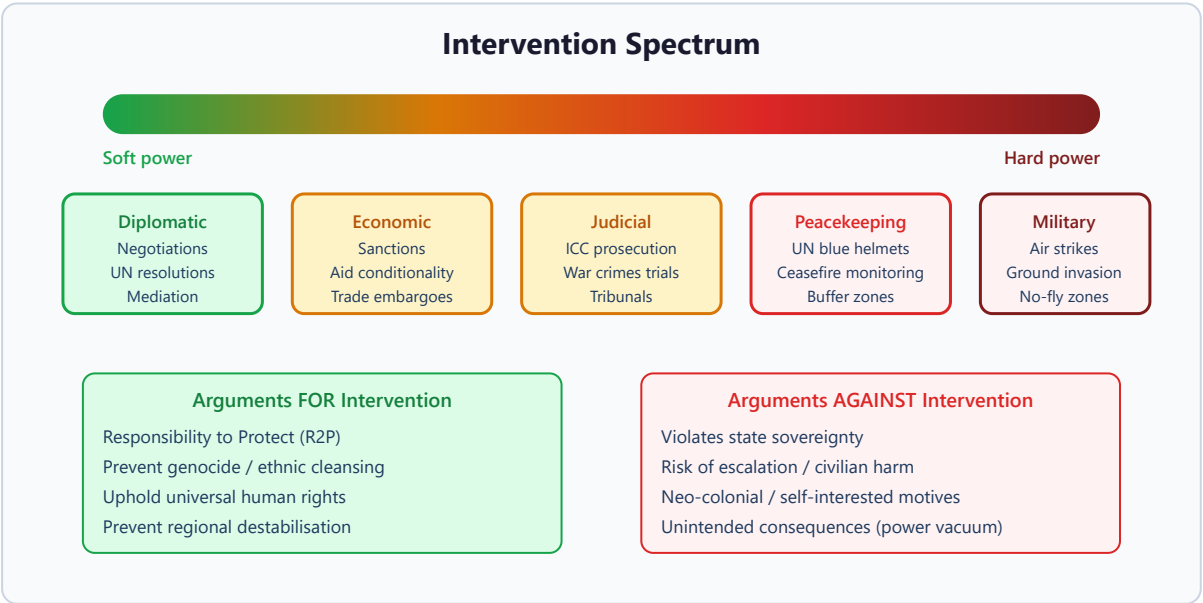
- **Conflict zones:** Syria (500,000+ deaths, 13 million displaced, chemical weapons use), Yemen (worst humanitarian crisis globally, 377,000+ deaths, 80% of population needing aid), Myanmar (Rohingya genocide), Ukraine (Russian invasion causing massive displacement and civilian casualties)
- **Authoritarian regimes:** North Korea (political prison camps holding 80,000–120,000 people, forced labour, severe restrictions on all freedoms), Eritrea (indefinite military conscription amounting to forced labour, no press freedom, no elections), China (mass surveillance, social credit system, restrictions on religious and ethnic minorities)
- **Gender-based violations:** Afghanistan (Taliban restrictions on women's education, employment, movement, and dress since 2021 — the only country banning girls from secondary school), FGM in parts of Africa and the Middle East, forced marriage affecting 12 million girls annually
- **Ethnic/religious persecution:** Rohingya in Myanmar (ethnic cleansing since 2017), Uyghurs in China (mass detention, forced labour, cultural suppression in Xinjiang), caste-based discrimination in India and Nepal

Case Study — Rohingya Crisis, Myanmar: Since August 2017, over **740,000** Rohingya Muslims have fled Myanmar (Burma) to neighbouring Bangladesh following a military offensive described by the UN as "a textbook example of ethnic cleansing." The Rohingya have been denied citizenship since the 1982 Citizenship Law, facing restrictions on movement, education, marriage, and healthcare. The 2017 military operations involved mass killings, widespread sexual violence (used systematically as a weapon of war), and the burning of hundreds of villages (satellite imagery documented the

destruction of over 350 villages). Over 1 million Rohingya now live in the world's largest refugee camp complex at Cox's Bazar, Bangladesh, in overcrowded conditions with limited access to education, healthcare, and livelihood opportunities. The crisis raises fundamental questions about: (1) state sovereignty vs the international **Responsibility to Protect (R2P)**, (2) the effectiveness of the UN Security Council (vetoes by China and Russia have blocked binding resolutions against Myanmar), (3) the capacity and willingness of neighbouring countries to absorb large refugee populations, (4) the role of social media in inciting violence (Facebook was used to spread anti-Rohingya hate speech), and (5) the slow pace of international justice (the ICJ ordered Myanmar to prevent genocide in 2020 but enforcement remains minimal).

6. Geopolitical Intervention

Geopolitical intervention: Actions taken by external actors (states, military alliances, IGOs such as the UN) to influence the internal affairs of another sovereign state, ranging from diplomatic pressure at the soft end to military invasion at the hard end. Interventions may be motivated by humanitarian concerns (protecting civilians from atrocities), strategic interests (access to resources, containing rival powers), ideological goals (promoting democracy), or a combination. The legitimacy, effectiveness, and consequences of intervention are hotly debated.



The Responsibility to Protect (R2P)

Adopted by the UN World Summit in 2005, R2P establishes three pillars of responsibility:

1. **Pillar 1 (State responsibility):** Every state has the primary responsibility to protect its own populations from genocide, war crimes, ethnic cleansing, and crimes against humanity
2. **Pillar 2 (International assistance):** The international community should assist states in meeting this responsibility through capacity building, early warning, and diplomacy
3. **Pillar 3 (Collective action):** If a state is **manifestly failing** to protect its population (or is itself the perpetrator), the international community should take **collective action** through the UN Security Council, which may include sanctions, peacekeeping, or, as a last resort, military intervention

R2P has been invoked in Libya (2011), Côte d'Ivoire (2011), and South Sudan (2013), but was notably NOT applied in Syria despite widespread atrocities. This selective application is a major criticism — R2P is only invoked when it aligns with the geopolitical interests of the P5 (permanent UN Security Council members: USA, UK, France, Russia, China). Russia and China's veto power has repeatedly blocked action in Syria and Myanmar.

Case Study — Libya (2011): NATO intervention in Libya was authorised by UN Security Council Resolution 1973 to **protect civilians** in Benghazi during the civil uprising against Colonel Muammar Gaddafi. A no-fly zone was imposed, followed by air strikes that supported rebel forces. **Outcome:** The immediate objective of protecting Benghazi was achieved, and Gaddafi was overthrown and killed. **However**, critics argue the intervention: (1) exceeded its mandate (regime change rather than civilian protection, undermining the legitimacy of R2P for future cases), (2) resulted in state collapse and a power vacuum, (3) enabled the rise of militia rule, with two rival governments and ongoing civil war, (4) contributed to a migrant crisis as Libya became a transit hub for people attempting to cross the Mediterranean to Europe (with horrific conditions in Libyan detention centres), and (5) the spread of weapons from Libyan arsenals destabilised the Sahel region (contributing to conflict in Mali, Niger, and Nigeria). Libya illustrates the law of **unintended consequences** and the critical importance of post-intervention planning.

Case Study — Syria (2011–present): The Syrian Civil War, which began with protests during the Arab Spring, has caused over **500,000 deaths** and displaced **13+ million people** (6.8 million internally, 6.8 million refugees). Despite widespread atrocities including documented chemical weapons attacks on civilians (Ghouta 2013, Khan Shaykhun 2017, Douma 2018) and the systematic targeting of hospitals and civilian infrastructure, direct military intervention to protect civilians was blocked by **Russian and Chinese vetoes** in the UN Security Council (Russia vetoed 16+ resolutions

on Syria). Limited US-led air strikes targeted chemical weapons facilities (2017, 2018) but did not aim to change the conflict's trajectory. Russia and Iran provided military support to Assad's government, while the USA and allies supported some opposition groups. Syria demonstrates how **geopolitical competition** (Russian strategic interests in its naval base at Tartus, Western reluctance after the Iraq and Libya experiences) can prevent humanitarian intervention even in the face of the most severe human rights violations, effectively rendering R2P powerless when P5 interests conflict.

7. Role of IGOs and NGOs

Intergovernmental Organisations (IGOs)

Organisation	Role	Strengths	Limitations
WHO	Sets global health standards and guidelines; coordinates pandemic preparedness and response; provides technical assistance; disease surveillance; eradication programmes	Global authority and technical expertise; successfully eradicated smallpox (1980); near-eradication of polio; coordinated COVID-19 response; sets Essential Medicines List	Dependent on voluntary member state funding (creating donor influence); limited enforcement power (can only recommend, not compel); political pressures (e.g. controversy over Taiwan's exclusion, delay in declaring COVID-19 a pandemic)
UN Security Council	Authorises peacekeeping missions, economic sanctions, and military intervention. Primary responsibility for maintaining international peace and security	Legitimate authority under international law (Chapter VII); can authorise binding resolutions; deploy peacekeeping forces (currently ~87,000 personnel in 12 missions)	**P5 veto power** paralyses action when permanent members' interests are at stake (Russia blocked Syria resolutions; China shields Myanmar). Geopolitical interests routinely override humanitarian concerns. Selective application of R2P undermines credibility

ICC (International Criminal Court)	Prosecutes individuals (not states) for genocide, war crimes, crimes against humanity, and the crime of aggression	Deters future atrocities (although deterrence is debated); delivers justice for victims; prosecuted figures like Thomas Lubanga (Congo), Al-Bashir (Sudan); issued arrest warrant for Putin (2023)	USA, Russia, China, India not members (limits reach); limited enforcement (relies on states to arrest suspects); accused of geographic bias (most cases from Africa); slow proceedings (years to complete)
World Bank / IMF	Fund health infrastructure, education, and development in LICs and MICs; provide loans and technical assistance	Significant financial resources ($100+ billion in annual lending); global reach; fund vaccination programmes, health system strengthening	Historical structural adjustment conditions (SAPs) forced governments to cut public health spending and introduce user fees, worsening health outcomes for the poorest. Western-dominated governance structures. Debt burden on recipient countries

Non-Governmental Organisations (NGOs)

NGO	Focus	Approach and Impact
MSF (Medecins Sans Frontieres)	Emergency medical care in conflict zones, epidemic outbreaks, and natural disasters. Nobel Peace Prize 1999	Neutral, impartial, independent medical assistance. Operates in 70+ countries. Provides healthcare where governments cannot or will not. Also speaks out publicly about neglected crises and human rights abuses witnessed in the field (*temoignage* / bearing witness)
Amnesty International	Human rights research, documentation, and advocacy worldwide	Documents abuses through on-the-ground research; campaigns for prisoners of conscience (people imprisoned for peacefully expressing their views); lobbies governments; produces annual State of the

		World's Human Rights report. Influenced the creation of the ICC
Oxfam	Poverty, inequality, and humanitarian response	Emergency relief (food, water, shelter); long-term development programmes (livelihoods, education, water/sanitation); advocacy for policy change (campaigns on tax justice, climate change, inequality). Challenged by internal scandals (Haiti, 2018)
Human Rights Watch	Investigating and reporting human rights abuses globally	Rigorous research-based advocacy using field investigations, interviews, and satellite imagery. Naming-and-shaming strategy to pressure governments. Legal advocacy in national and international courts. Influential in shaping media coverage and policy
Bill & Melinda Gates Foundation	Global health, poverty, education	Largest private foundation ($75+ billion endowment). Major funder of vaccine development (Gavi), malaria research, and agricultural innovation. Criticised for excessive influence over global health priorities and lack of democratic accountability

Exam Tip: Edexcel questions often require you to **evaluate the effectiveness** of interventions. Use specific case studies and consider multiple perspectives: (1) Was the intervention justified (legal basis, humanitarian need)? (2) Did it achieve its stated objectives? (3) Were there unintended consequences (state collapse, refugee flows, regional instability)? (4) Who benefited and who was harmed? (5) What does this tell us about the tension between state sovereignty and the international responsibility to protect? Always consider power dynamics: why are some crises intervened in (Libya) while others are not (Syria, Rohingya)? The answer usually lies in the geopolitical interests of the P5 powers.

8. Global Governance of Health

MDGs to SDGs

The **Millennium Development Goals (MDGs, 2000–2015)** included specific health targets. Progress was significant but uneven:

- Child mortality (U5MR) fell by over 50% globally (from 90 to 43 per 1,000 live births)
- New HIV infections declined by 40%; 15 million people accessing ART by 2015
- Maternal mortality fell by 45%, short of the 75% target (Sub-Saharan Africa lagged furthest behind)
- Malaria deaths fell by 60%, partly due to insecticide-treated bed nets (ITNs) funded by the Global Fund

The **Sustainable Development Goals (SDGs, 2015–2030)** include SDG 3 (Good Health and Well-being) with broader targets covering: maternal mortality (<70 per 100,000 by 2030), communicable diseases (end AIDS, TB, malaria epidemics), NCDs (reduce premature mortality by one-third), mental health, substance abuse, universal health coverage (UHC), and access to essential medicines and vaccines. Progress is currently off-track for most SDG 3 targets, exacerbated by COVID-19 disruption.

Pharmaceutical TNCs and Access to Medicines

Transnational pharmaceutical corporations play a crucial but deeply contested role in global health:

- **Patent protection (TRIPS Agreement):** The WTO's TRIPS agreement protects pharmaceutical patents for **20 years**, maintaining high prices and limiting access in LICs. A year's supply of ART for HIV cost $10,000+ per patient when first introduced, falling to ~$75 with generic competition. The tension between intellectual property rights (incentivising innovation) and access to essential medicines is a central debate in global health governance
- **Generic medicines:** India is the "pharmacy of the developing world," producing ~70% of global generic medicines. Generic ARVs for HIV treatment in Africa cost a fraction of branded versions. The Doha Declaration (2001) affirmed that TRIPS should not prevent countries from protecting public health and allows compulsory licensing (governments can produce or import generics for public health emergencies)
- **Neglected tropical diseases (NTDs):** Pharmaceutical companies invest less in diseases affecting mainly poor populations (sleeping sickness, Chagas disease, leishmaniasis, river blindness) because there is limited profit potential. This is the "90/10 gap" — historically, 90% of global health research spending targeted diseases affecting only 10% of the global disease burden (affluent populations)
- **Vaccine inequality (COVID-19):** The pandemic starkly exposed global inequalities in access to health technologies. By mid-2021, wealthy nations had vaccinated over 50% of their populations while many African nations remained below 2%. Vaccine nationalism (wealthy countries securing bilateral deals directly with manufacturers, hoarding surplus doses) undermined multilateral efforts

Case Study — COVAX Initiative: COVAX (COVID-19 Vaccines Global Access) was established as a global mechanism to ensure equitable vaccine distribution, co-led by Gavi (the Vaccine Alliance), CEPI (Coalition for Epidemic Preparedness Innovations), and the WHO. It aimed to deliver **2 billion doses** to LICs and MICs by end of 2021. However, COVAX fell far short of its targets due to: (1) **vaccine nationalism** — wealthy countries negotiated bilateral deals with manufacturers, securing enough doses for their own populations many times over, leaving COVAX unable to compete, (2) **export bans** by producer countries (e.g. India temporarily banned vaccine exports during its devastating second wave), (3) **production shortages** and supply chain bottlenecks, (4) **intellectual property barriers** — proposals by India and South Africa for a temporary TRIPS waiver on COVID vaccines were initially blocked by wealthy nations protecting pharmaceutical company interests. By end of 2021, COVAX had delivered only ~950 million of its 2 billion dose target. This experience highlighted the enduring challenge of translating global health governance principles into effective action when they conflict with national self-interest and corporate profit.

Exam Tip: When writing about global health governance, demonstrate **synoptic thinking** by linking this topic to other areas of your course: globalisation (TNCs, trade rules), development (aid, debt, governance), superpowers (geopolitical alliances shaping health policy), and migration (health of refugees and migrants). For example, discuss how TNC power, intellectual property rules, vaccine nationalism, and geopolitical alliances all intersect to shape who has access to medicines and health technologies. The best answers recognise that health is not merely a medical issue but a deeply political one, shaped by power, inequality, and governance at multiple scales.

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